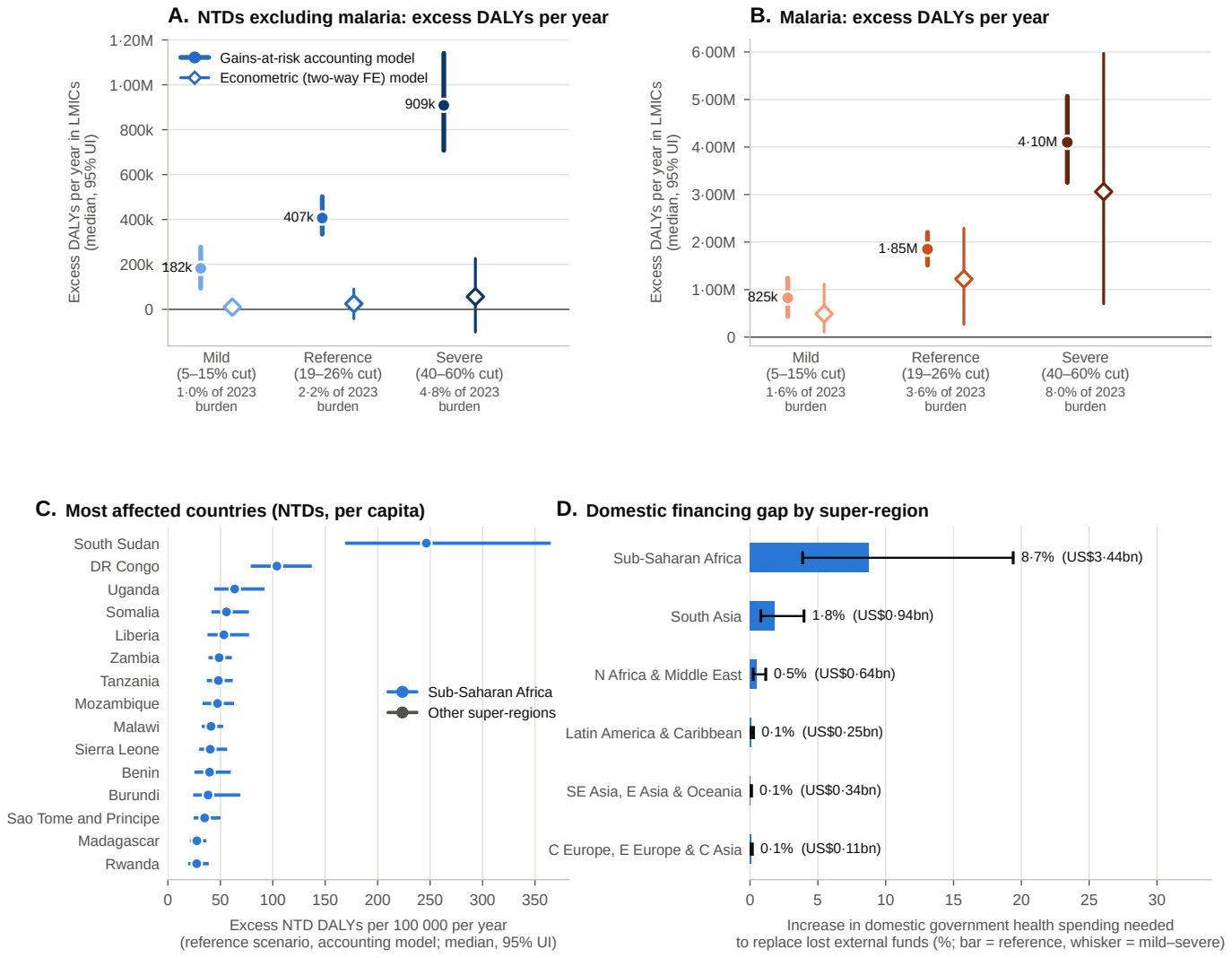
A. NTDs excluding malaria: excess DALYs per year
Gains-at-risk accounting model
Econometric (two-way FE) model
Excess DALYs per year in LMICs (median, 95% UI)
1·20M
1·00M
800k
600k
400k
200k
0
909k
407k
182k
Mild (5–15% cut) 1·0% of 2023 burden
Reference (19–26% cut) 2·2% of 2023 burden
Severe (40–60% cut) 4·8% of 2023 burden
B. Malaria: excess DALYs per year
Excess DALYs per year in LMICs (median, 95% UI)
6·00M
5·00M
4·00M
3·00M
2·00M
1·00M
0
4·10M
1·85M
825k
Mild (5–15% cut) 1·6% of 2023 burden
Reference (19–26% cut) 3·6% of 2023 burden
Severe (40–60% cut) 8·0% of 2023 burden
C. Most affected countries (NTDs, per capita)
South Sudan
DR Congo
Uganda
Somalia
Liberia
Zambia
Tanzania
Mozambique
Malawi
Sierra Leone
Benin
Burundi
Sao Tome and Principe
Madagascar
Rwanda
Sub-Saharan Africa
Other super-regions
0
50
100
150
200
250
300
350
Excess NTD DALYs per 100 000 per year (reference scenario, accounting model; median, 95% UI)
D. Domestic financing gap by super-region
Sub-Saharan Africa
8·7% (US$3·44bn)
South Asia
1·8% (US$0·94bn)
N Africa & Middle East
0·5% (US$0·64bn)
Latin America & Caribbean
0·1% (US$0·25bn)
SE Asia, E Asia & Oceania
0·1% (US$0·34bn)
C Europe, E Europe & C Asia
0·1% (US$0·11bn)
0
5
10
15
20
25
30
Increase in domestic government health spending needed to replace lost external funds (%; bar = reference, whisker = mild–severe)